HealthConnect Rwanda
Executive Summary

The immediate decision is to start recording appointment outcomes. HealthConnect can already review booking workload, payments and patient feedback. To know whether reminders or easier rescheduling would help, it first needs a reliable record of who attended and who missed a visit.

What the Data Shows

Appointments. The extract contains 5,000 bookings across 4,443 patients, with 1,235 to 1,271 bookings per quarter in the 2026 extract. The file does not record attendance outcomes, so these counts cannot establish a no-show rate.

Finance. Collections were RWF 8.39 billion against RWF 9.31 billion billed. Only 8.28% of payment value links to a recorded booking, limiting facility comparisons. The separate monthly summary reports RWF 1.13 billion in revenue.

Patient feedback. The average rating was 2.99 out of 5 across 21,064 responses. Only 8.34% of responses link to a booking, so the overall score can guide a review of patient support while facility rankings need better coverage.

Recommended Next Step

Record whether each appointment was attended, missed, cancelled or rescheduled, and check how consistently staff capture outcomes. Once the clinic has a reliable baseline, compare a small reminder or rescheduling pilot with the current process. Track the no-show rate, completed reschedules and patient feedback. If the pilot includes an AI assistant, give it approved scheduling information and route unresolved requests to clinic staff.

Evidence and Scope

The Week 7 checks showed that a join through bed records could inflate the appointment count from 5,000 to 50,000 rows. Using one lookup row per department restored the count in the test. The Week 8 review recalculated source figures. Live Power BI checks remain pending.

Sources: Joyce's Week 7 Testing and Refinement Report, Week 8 Analytics Report, and the underlying HealthConnect appointment, finance and feedback extracts. RWF follows the project's reporting assumption.